Maternal, Labor, Developmental, and Family Assessment

NURSING · nursing-foundations · BoardPrep lesson v5

Official examination scope

PRC Resolution No. 10 s. 2025 and the August 2026 NLE program define examination scope, not topic weights. This lesson teaches a bounded maternal, postpartum, developmental, and family assessment foundation from the references below. Open RN and U.S. professional materials support general educational principles only; apply Philippine law, scope of practice, current clinical orders, referral pathways, and facility policy. This study material is not a clinical directive or proof of complete NP coverage.

Learning objectives

- Recognize concerning maternal symptoms and prioritize timely assessment and escalation without
- diagnosing from a brief vignette.
- Assess postpartum changes and feeding concerns through observation, baseline, symptoms, and
- the person’s goals.
- Interpret development as a contextual trajectory and distinguish screening concern from
- diagnosis.
- Use respectful family partnership and teach-back to make discharge teaching understandable and
- actionable.
- Offer preference-based labor comfort options while separating comfort support from clinical
- assessment and progress decisions.

1. Treat maternal warning cues as assessment priorities

Pregnancy can include expected discomfort, but severe headache, visual disturbance, upper-abdominal pain, bleeding, breathing difficulty, seizure, or marked deterioration must not be dismissed as routine. A cluster of severe headache, vision change, and upper-right abdominal pain is concerning and warrants prompt assessment and urgent escalation under the local maternity pathway. Gather timing, associated symptoms, relevant observations, and fetal or maternal concerns within role; communicate clearly and reassess while help is arranged. Recognition is not diagnosis. The WHO symptom reference supports concern recognition, while management thresholds and treatment remain with current Philippine guidance and authorized clinicians.

Sources for this section

Section reference: Pre-eclampsia — World Health Organization — https://www.who.int/news-room/fact-sheets/detail/pre-eclampsia
Section reference: WHO Recommendations for Care of the Preterm or Low-Birth-Weight Infant and Postnatal Care — World Health Organization — https://www.who.int/news/item/30-03-2022-who-urges-quality-care-for-women-and-newborns-in-critical-first-weeks-after-childbirth

2. Reassess postpartum symptoms rather than normalizing them

The postpartum period does not remove risk. Ask what changed, when it started, how severe it is, and whether symptoms are worsening; compare observations with the person's baseline and current recovery context. Severe headache with visual change needs urgent assessment; unilateral leg pain together with new shortness of breath is a time-sensitive emergency cue. Do not diagnose a clot or hypertensive disorder from symptoms alone, massage a painful leg, delay for routine follow-up, or substitute a home remedy. Activate the appropriate local maternity/emergency pathway and continue focused reassessment. Do not import U.S. surveillance schedules or protocol thresholds from the textbook. CDC's U.S. public guidance also describes postpartum clot risk and limb or breathing symptoms; it informs recognition only and does not replace Philippine or facility pathways.

Sources for this section

Section reference: 11.5 Applying the Nursing Process and Clinical Judgment Model to Postpartum Care — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/11-5-applying-the-nursing-process-and-the-clinical-judgment-model-to-postpartum-care/
Section reference: WHO Recommendations for Care of the Preterm or Low-Birth-Weight Infant and Postnatal Care — World Health Organization — https://www.who.int/news/item/30-03-2022-who-urges-quality-care-for-women-and-newborns-in-critical-first-weeks-after-childbirth
Section reference: Pre-eclampsia — World Health Organization — https://www.who.int/news-room/fact-sheets/detail/pre-eclampsia
Section reference: Understanding Your Risk for Blood Clots with Pregnancy — U.S. Centers for Disease Control and Prevention — https://www.cdc.gov/blood-clots/risk-factors/pregnancy.html

3. Interpret development across context and time

Developmental assessment combines observed skills, caregiver observations, health history, environment, opportunities, and change over time. One missed or demonstrated task is not a diagnosis. Ask what the child usually does, whether a skill has changed or been lost, and what the caregiver notices across settings. A screening concern is a reason for a closer assessment or referral through the local child-health pathway, not proof of a disorder. Use current Philippine developmental guidance and qualified assessment; the source textbook's U.S. screening schedules are not reproduced here.

Sources for this section

Section reference: 13.4 Nursing and Medical Care for the Developing Child — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/13-4-nursing-and-medical-care-for-the-developing-child/

4. Build a safe, respectful family plan

Family-centered care asks who the parent wants involved, what beliefs and routines matter, and what safety questions need discussion. For feeding pain, clicking, or short feeds, observe a feed and assess positioning, latch, comfort, and transfer before recommending a change; refer when findings are outside nursing scope. Offer understandable, evidence-informed choices without shaming or assuming one family practice. For discharge teaching, use plain language, ask the caregiver to explain the plan in their own words, and reteach or change the format when understanding is incomplete. Teach-back checks the explanation, not the learner's intelligence. Feeding guidance and local newborn warning-sign pathways need nurse educator confirmation.

Sources for this section

Section reference: 11.5 Applying the Nursing Process and Clinical Judgment Model to Postpartum Care — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/11-5-applying-the-nursing-process-and-the-clinical-judgment-model-to-postpartum-care/
Section reference: WHO Recommendations for Care of the Preterm or Low-Birth-Weight Infant and Postnatal Care — World Health Organization — https://www.who.int/news/item/30-03-2022-who-urges-quality-care-for-women-and-newborns-in-critical-first-weeks-after-childbirth
Section reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

5. Support coping and choice during labor

Labor support combines the person's preferences with ongoing maternal and fetal assessment. Ask what helps, explain available choices in plain language, and offer a chosen companion when permitted by the setting. WHO recommends relaxation options such as breathing for healthy pregnant women requesting pain relief, depending on preference, and mobility or upright positions for women at low risk; these are options, not mandatory techniques or treatment for abnormal findings. A person may rest when contractions ease if that is comfortable and consistent with monitoring. For example, if someone in early labor asks how to conserve energy, discuss breathing or another preferred relaxation approach, rest between contractions, and position changes as tolerated, while clarifying which symptoms or local instructions require review. Comfort support does not establish labor progress, and a concerning maternal or fetal change requires assessment and the current local maternity pathway.

Sources for this section

Section reference: 10.5 Fetal Heart Rate Monitoring — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/10-5-fetal-heart-rate-monitoring/
Section reference: WHO recommendations: Intrapartum Care for a Positive Childbirth Experience — Summary List of Actions — World Health Organization — https://cdn.who.int/media/docs/default-source/maternal-health/ipc.pdf?sfvrsn=dc30fc00_8

6. Interpret labor progress in context

Cervical change and labor duration vary between individuals, so a single rate is not a complete assessment. WHO states that a 1 cm/hour threshold alone is inaccurate for identifying risk and that slower dilation alone should not routinely trigger intervention; its recommendations also distinguish latent from active labor and condition some guidance on reassuring maternal and fetal status. In practice, trend the findings with contraction pattern, maternal condition, fetal assessment, the person's experience, and the relevant care plan, then communicate changes to the authorized maternity team. A worked case with slow change but no other concerning finding does not license a nurse to diagnose dystocia or accelerate labor. The WHO recommendations are global and inform local policy; they do not replace Philippine facility pathways, clinician judgment, or current orders.

Sources for this section

Section reference: WHO recommendations: Intrapartum Care for a Positive Childbirth Experience — Summary List of Actions — World Health Organization — https://cdn.who.int/media/docs/default-source/maternal-health/ipc.pdf?sfvrsn=dc30fc00_8

Escalate pregnancy warning cues

Question: A pregnant client reports severe headache, vision changes, and upper-right abdominal pain.
1. Treat the combination as a warning cue, not as a confirmed diagnosis.

2. Assess promptly within role, obtain the relevant current observations, and note timing and change.
3. Use the local maternity escalation route and communicate the symptom cluster without delay.
Answer: Urgent assessment and escalation take priority over reassurance or routine scheduling.
Check: No threshold, drug, or treatment is inferred from a symptom vignette.

Compare postpartum risk cues

Question: A postpartum client reports severe headache and new visual disturbance.
1. Ask about onset, severity, associated symptoms, and relevant change.
2. Assess within role and recognize that serious complications can arise after birth.
3. Escalate through the current maternity pathway and reassess while help is arranged.
Answer: Do not assume sleep loss; prompt assessment and escalation are needed.
Check: The lesson does not specify a treatment or local vital-sign cutoff.

Observe feeding before advising

Question: A parent reports severe nipple pain and the infant makes clicking sounds during short feeds.
1. Ask permission to observe a feed and clarify the parent's goal.
2. Observe positioning, latch, comfort, and signs of transfer; assess for injury or other concerns.
3. Offer individualized support or refer when assessment exceeds role.
Answer: Direct observation guides teaching; pain alone does not establish infection or mean feeding must stop.
Check: Advice follows observed cues and the family's informed preferences.

Use teach-back at transition

Question: A caregiver cannot explain the newborn danger signs after discharge teaching.
1. Pause and ask which part is unclear without blame.
2. Explain again in shorter language or another format, focusing on the action and where to seek help.
3. Ask the caregiver to describe the plan in their own words; clarify remaining gaps.
Answer: Reteach and check understanding before transition according to local discharge policy.
Check: Providing a handout alone is not evidence that the safety message was understood.

Read developmental information over time

Question: A caregiver asks whether one missed milestone proves a developmental disorder.
1. Ask about the child's history, observed skills, changes, and environment.
2. Compare a pattern with an appropriate developmental reference rather than a single task or sibling.
3. Explain that a concern merits screening or qualified follow-up, not a diagnosis by the nurse from one observation.
Answer: Use a contextual trajectory and refer through the current child-health pathway.
Check: No U.S. screening timetable or diagnosis is adopted.

Choose labor comfort supports

Question: A client in early labor asks how to conserve energy and manage discomfort.
1. Ask what the client prefers and whether the maternity team has identified any reason to limit movement or oral intake.
2. Offer breathing or another chosen relaxation approach for pain relief, rest between contractions if comfortable, and position changes as tolerated.
3. Continue the assigned maternal and fetal observations; report new concerning symptoms through the local pathway.
Answer: Use preference-based comfort measures while keeping assessment and escalation active.

Check: The comfort plan is not used to diagnose progress or delay review of a concerning change.

Common misconceptions

- A concerning symptom cluster is not a diagnosis, but uncertainty is not a reason to delay
- urgent assessment.
- Postpartum symptoms are not automatically normal recovery; assess the change and escalate
- within local pathways.
- A developmental screen is not a diagnosis, and one task is not the whole developmental
- trajectory.
- Feeding pain alone does not prove infection or require stopping; observe and assess before
- advising.
- Teach-back evaluates how clearly the plan was explained, not whether a caregiver is capable.
- Breathing, rest, and position options are preference-based comfort supports; they do not prove
- that labor is normal or replace maternal/fetal assessment.
- A slower cervical dilation rate alone is not a universal diagnosis or routine indication for
- intervention; use the whole clinical context and current local pathway.

Practice patterns

- For maternal symptoms, identify warning cues, assess promptly, and escalate without claiming a
- diagnosis.
- For postpartum concerns, compare with baseline and use the local urgent pathway.
- For child development, interpret a pattern over time and distinguish screening from diagnosis.
- For feeding and discharge teaching, observe, collaborate, and verify understanding.
- For labor comfort, select preference-based coping measures while continuing indicated maternal
- and fetal assessment.
- For labor-progress questions, reject a single rate as a universal trigger and interpret trends
- with maternal/fetal status and local pathways.

Official scope references

Official scope: Enhanced Tables of Specifications for the Subjects in the Nurses Licensure Examination — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/2025-10%20%20published%20Enhanced%20TOS%20nuse.pdf
Official scope: Nursing Licensure Examination Program, August 2026 — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/exam%20progam%20aug%202026%20nle%20(nursing)%20signed.pdf

Instructional references

Instructional reference: Pre-eclampsia — World Health Organization — https://www.who.int/news-room/fact-sheets/detail/pre-eclampsia
Instructional reference: 11.5 Applying the Nursing Process and Clinical Judgment Model to Postpartum Care — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/11-5-applying-the-nursing-process-and-the-clinical-judgment-model-to-postpartum-care/
Instructional reference: WHO Recommendations for Care of the Preterm or Low-Birth-Weight Infant and Postnatal Care — World Health Organization — https://www.who.int/news/item/30-03-2022-who-urges-quality-care-for-women-and-newborns-in-critical-first-weeks-after-childbirth
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Instructional reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/
Instructional reference: 10.5 Fetal Heart Rate Monitoring — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/10-5-fetal-heart-rate-monitoring/
Instructional reference: Understanding Your Risk for Blood Clots with Pregnancy — U.S. Centers for Disease Control and Prevention — https://www.cdc.gov/blood-clots/risk-factors/pregnancy.html
Instructional reference: WHO recommendations: Intrapartum Care for a Positive Childbirth Experience — Summary List of Actions — World Health Organization — https://cdn.who.int/media/docs/default-source/maternal-health/ipc.pdf?sfvrsn=dc30fc00_8
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